

Clinical Brief

Patient ID: P004

Alert Summary

trigger: Blood pressure 168/102 mmHg detected by Omron BP Monitor, exceeding threshold of 140/90 mmHg

severity_class: ELEVATED

severity_score: 6

timestamp: 2026-05-14T11:00:00Z

Patient Snapshot

patient_id: P004

demographics: Demographics not available (missing from EHR)

active_conditions

- Hypertension (active, onset unknown)

current_medications

- Amlodipine (dose and frequency unknown)

contextual_analysis: The patient is a recent transfer with a self-reported history of hypertension and is currently taking Amlodipine, though dose and frequency are unknown (EHR). The elevated BP reading of 168/102 mmHg is the first documented value at this facility, with no prior readings or trends available (EHR). The patient reports no missed doses of Amlodipine and expresses concern about the BP reading and uncertainty regarding medication dosing (Anamnesis). Mild, dull, occipital headache was reported on the day of the elevated BP (Anamnesis).

risk_assessment: This elevated BP may suggest suboptimal hypertension control or possible medication underdosing, especially in the context of high stress and incomplete medication information. The presence of mild headache could indicate a symptomatic response to elevated BP, but there are no reports of chest pain or other acute symptoms.

recommended_actions

- - action: Consider confirming current Amlodipine dose and frequency with patient and prior provider when records arrive
 - confidence: 0.9
 - source: EHR + Anamnesis
- - action: Suggest repeat BP measurement within 24-48 hours to assess for persistent elevation
 - confidence: 0.85
 - source: Alert
- - action: Recommend monitoring for new or worsening symptoms (e.g., chest pain, vision changes, neurological deficits)
 - confidence: 0.8
 - source: Anamnesis

uncertainties_and_gaps

- Patient age and gender not documented
- No prior blood pressure readings or trends available
- Amlodipine dose and frequency unknown
- No information on other antihypertensive medications
- No recent laboratory results
- No transfer records or comorbidity details received
- Smoking status and alcohol use not documented
- No explicit report of chest pain or other acute symptoms